NCCP 00414 · VERSION 5A

Palbociclib with endocrine therapy

Take palbociclib for 21 days, then have 7 days without palbociclib. The aromatase-inhibitor tablet continues every day, or fulvestrant injections follow their separate schedule. The week off applies to palbociclib, not automatically to the hormone treatment.

> Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Menopause and ovarian-suppression requirements if relevant

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Low neutrophils and infection risk

☐ Mouth soreness

☐ Hair thinning

☐ Hot flushes and other hormone-related symptoms

☐ Fulvestrant: injection-site pain and bruising

☐ Fatigue

☐ Nausea or diarrhoea

☐ Rare but serious interstitial lung disease / pneumonitis

☐ AI: joint symptoms and bone loss

☐ Partner-specific liver, interaction and pregnancy considerations

The same low count can lead to a different action at Day 1 and Day 15 of the first two cycles. Dose decisions also depend on fever/infection, recovery, severity and current dose. A clinician must confirm a restart; the patient timeline is an explanation, not permission to self-adjust treatment. Do not use during pregnancy or breastfeeding. EMA specifies effective contraception during treatment and for at least 3 weeks after the last dose for women and 14 weeks for men; the endocrine partner may require a longer interval. Discuss fertility preservation before treatment when relevant.

Individual benefit, alternatives and concerns

Agreed checks and next review

Before the first dose: FBC, kidney and liver bloods; review other medicines, pregnancy possibility, receptor results, menopausal status and the exact endocrine partner. Confirm ovarian suppression when relevant. FBC on Day 1 and Day 15 of cycles 1–2, then Day 1 from cycle 3. Kidney/liver bloods before each cycle. If counts are low, extra blood tests and a pause may be needed. Only after no worse than grade 1/2 neutropenia in the first 6 cycles may FBC frequency reduce to every 3 months before a cycle, with additional tests when clinically indicated.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.